



For Staff

Student Expresses Suicidal Thoughts

Responding when a student says they want to die, or shows they may be thinking about suicide: immediate safety, asking directly, and getting the right help

Content Warning This guide covers suicidal thoughts, self-harm and suicide.

This is reference guidance, not a training course. Use it to check how to respond, or to prepare for a conversation. It does not replace the school's safeguarding training, policy or the DSL's advice on a specific case.

It contains discussion of suicidal thoughts, self-harm, suicide attempts and deaths by suicide, without any description of methods. Some of it may be difficult to read, particularly if it connects with your own experience or that of someone close to you.

- You can read it in sections, take a break, or step away at any point.
- If this raises something personal, you can speak in confidence to your line manager, a senior colleague you trust, or the school's Employee Assistance Programme, if it has one, or **Education Support** (free, confidential, 24/7 on 08000 562 561). **Samaritans** is free, any time, on 116 123.
- If you have a question about how this guidance applies to a situation you are dealing with, speak to your school's Designated Safeguarding Lead (DSL).

If reading this makes you worried about a child now, do not wait until you have finished reading: speak to the DSL or a deputy straight away. If a child is in immediate danger, call 999.

Immediate danger or possible risk to life: call 999 when it is needed, stay with the child, and summon the DSL or a deputy. Don't delay action to work through the steps. The 90-second Pause is for once immediate safety is secured, to stop an impulsive or investigative response. **Go to the Quick Reference Card.**

Why This Matters

Suicidal thoughts and self-harm are more common among young people than many adults realise. In a UK study of more than 10,000 17-year-olds, **7.4% said they had ever attempted suicide** and **24.1% had self-harmed in the previous year** ([Patalay and Fitzsimons, 2021](#)). Around half of young people under 25 who died by suicide had previously self-harmed ([NCISH, 2017](#)).

KCSIE 2026 is clear that mental health problems can, in some cases, develop into safeguarding concerns, including self-harm, suicidal ideation or risk of suicide (paragraph 47). If staff feel a child is in danger, they should call 999 or arrange for them to be taken to A&E immediately; if a child needs urgent mental health help that is not an emergency, staff can use NHS 111 and select the mental health option (paragraph 50). Suicide is preventable, and a calm, direct response from one adult can make the difference.

Scenario Overview

During an after-school exam revision session, a Year 11 student breaks down in tears. They are breathing fast, cannot settle, and say: "I can't do this anymore, I want to die."

What to Keep in Mind

- **Take every statement about wanting to die seriously**, even if it sounds like frustration, comes out in a panic, or the student later says they didn't mean it.
- **If there may be immediate risk, don't leave the student alone.** Stay with them and send for the DSL or a deputy. If a life is at risk, call 999.
- **Ask directly about suicide.** Asking does not put the idea in someone's head; it can protect them (Samaritans; YoungMinds).
- Suicidal thoughts can also show up **in writing**: a line in an assignment such as "I don't plan to still be here by the time I leave school" needs the same response, the same day.
- It may follow a **pattern** of withdrawal, poor sleep or falling work. A pattern can help explain what is happening, but **you don't need to wait for it to happen again** before telling the DSL.
- You are not expected to assess risk or provide therapy. Your job is to keep the student safe now, listen, and get them to the people who can help.
- Where the student is 18, see the note for the DSL under Notify.

How RESPOND Works in This Scenario

RESPOND is powered by **ACT: Active Intervention** (inaction is not neutral), **Contextual Safeguarding** (exam pressure, friendships and online spaces all play a part) and **Trauma-Informed Practice** (safety before emotion before thinking). The steps are taught in order but work as a **helix**: see *The RESPOND Helix* section. With immediate risk, you go straight to Notify and stay with the student while you do.

Step	RESPOND tool	In this scenario
● Recognise	4 Ps Context Scan; KCSIE warning signs	Take the words seriously; notice the pattern behind them
● Engage	Ask directly; TED questions	"Are you thinking about suicide?" Listen without judging
● Support	Safety check first; "Your calm is contagious"	Never alone if there may be immediate risk; medical help if needed
● Pause	A short Pause; Three Lenses	A breath, not a delay. Don't investigate or promise secrecy
● Offer	Model alternatives; if in doubt, N before O	Hope, options and people who can help
● Notify	DSL immediately where there is risk; 999 if a life is at risk	The DSL leads on mental health routes and parents
● Document	GOLD standard	Their exact words, and exactly what you did

RESPOND is not a checklist to complete in order. Immediate danger or possible risk to life: act first (999 when needed, stay with the child, summon the DSL or a deputy), then return to the steps.

Need the essentials? The *Quick Reference Card* at the end of this guide covers the immediate response and escalation on one page.

The One Chance Rule: a disclosure may be a young person's only attempt to seek help. Make it count.

The RESPOND Helix: How the Steps Move

RESPOND is taught in a line, Recognise to Document, because sequence gives structure under pressure. In practice it moves as a helix. The first dimension is the steps themselves: you may loop back, hold at a step, or go straight to Notify. The second is time and severity: each turn sits

higher as a concern develops. The third is context: a concern involving several children, places or platforms traces a wider spiral.

Relational and Systemic

For a child, safeguarding is **relational**: it happens through people they trust. For the adults around them, it is also **systemic**: the steps, records and shared decisions that make sure what a child tells one person reaches the people who can help.

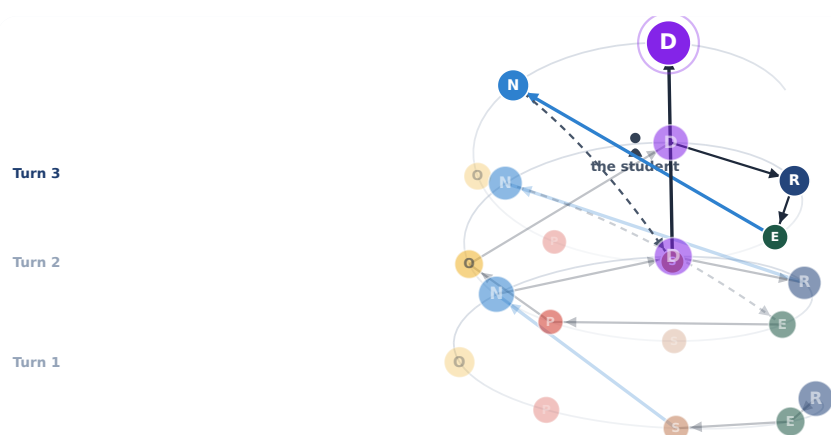
Relational is why children tell. Systemic is why telling works.

The RESPOND steps follow the same shape: Recognise, Engage and Support are relational; Pause is the bridge; Offer, Notify and Document are the systems steps. Neither works alone. Relationship without the system becomes secrecy, which is why staff never promise to keep a disclosure to themselves. The system without relationship becomes process that children do not trust enough to use. When you share information, be open with the child about what you are sharing and why.

See It Move

Two examples: a pattern of withdrawal that leads to a disclosure, and an immediate-risk moment that goes straight to Notify. Then see a wider spiral, where a friend and a group chat are involved, and what happens when steps are skipped.

This is a still image of the interactive RESPOND Helix. Open the web version of this guide to step through the examples; every example is also written out in full below.



▼ Read the helix demonstration as text

Sequence

How RESPOND is taught.

1. **The seven steps:** RESPOND is taught in this order, Recognise to Document, because sequence gives structure under pressure. Recognise, Engage and Support are the relational steps; Pause is the bridge; Offer, Notify and Document are the systems steps.
2. **But practice is not a line:** Suicidal thoughts often build quietly and then surface suddenly. You may loop back, hold at a step, or go straight to Notify when there is immediate risk. Choose an example to see how the steps move.

From withdrawal to disclosure

A Year 11 student over one exam term: first quiet, then a line in written work, then a breakdown in a revision session.

1. **Turn 1: something has changed:** A tutor notices a Year 11 student has stopped eating lunch with friends, looks exhausted and has missed two deadlines. Recognise: use the 4 Ps. Exams are close, and a grandparent died last term.
2. **Turn 1:** Engage: a quiet check-in. TED: "Tell me how things have been lately." The student says they are "just tired" and "stressed about mocks".
3. **Turn 1:** Support: the tutor says they can come back any time, and names one other adult they could go to. The door stays open.
4. **Turn 1:** Notify: the tutor passes the changes to the DSL through the school's usual system. You don't need a disclosure, and you don't need to wait for it to happen again.
5. **Turn 1:** Document: GOLD. What was seen, what was said, and the dates, so a pattern can be seen if there is one.
6. **Turn 2: in writing (a week later):** In an English assignment, the student writes: "I don't plan to still be here by the time I leave school." Because the first concern was recorded, the DSL can connect them.
7. **Turn 2: N before O:** Notify before anything else: the teacher tells the DSL the same day, before the student goes home. Written words get the same response as spoken ones.
8. **Turn 2:** Engage, as the DSL plans: a trusted adult asks directly, "Are you thinking about suicide?" The student says they have thought about it but are safe tonight. Use the script: "I believe you, and I'm taking what you've told me seriously. This is not your fault. I'm going to help keep you safe, and I'll explain what happens next."
9. **Turn 2:** Pause: Three Lenses. Risk: thoughts, no immediate plan known. Rights: to be heard and involved. Resources: the DSL, the counsellor, parents, NHS 111 option 2.
10. **Turn 2:** Offer: model alternatives. A safety plan made with the student, a key adult, and support with exam pressure. Parents told, as the DSL decides.
11. **Turn 2:** Document: the exact line from the assignment, the question asked and the answer, and the plan.
12. **Turn 3: the breakdown:** In an after-school revision session, the student breaks down, breathing fast, and says: "I can't do this anymore, I want to die."
13. **Turn 3:** Engage: the teacher stays calm, moves them to a quiet room with another adult nearby, breathes slowly with them, then asks directly about suicide and whether they have hurt themselves or taken anything.
14. **Turn 3: straight to Notify:** The student says they have been thinking about it all day. The teacher treats it as immediate risk: a colleague fetches the DSL now. The teacher does not leave the student.

5. **Turn 3:** Support: the teacher stays with them until the DSL takes over. The DSL seeks urgent mental health advice and contacts parents. You never leave a step behind.
6. **Turn 3:** Document: GOLD, as soon as is practicably possible, with the student's exact words and the times.

Immediate risk

A Year 10 student tells a teacher at the end of a lesson that they have taken something to hurt themselves. One turn, and it moves fast.

1. **Recognise:** "I've taken something. I didn't want to be here anymore." This is a medical emergency as well as a safeguarding one.
2. **Straight to Notify:** Go straight to Notify: call 999, even if the student seems well, and send a colleague for the first aider and the DSL. With immediate danger, 999 comes first.
3. **Support:** Support: stay with the student. Keep them calm and still, and keep talking to them. Don't leave them alone for a moment.
4. **Engage:** Engage: "You did the right thing telling me. Help is coming, and I'm staying with you." Pass anything the student says about what they took to the paramedics.
5. **Pause:** Pause: a breath, not a delay. Check your own state so your calm carries the room. Protect the student's dignity from others watching.
6. **Offer:** Offer, once the student is with paramedics and the DSL: explain what happens next, and that they are not in trouble.
7. **Document:** Document: GOLD, once the student is safe. Exact words, times, who was called and when. Then ask the DSL for a debrief: this was frightening for you too.

Wider spiral

Suicidal thoughts are rarely held by one student alone. One student, a friend who knows, and a group chat.

1. **Three strands:** A Year 9 student messages a friend late at night saying they want to die, and posts a dark message in a group chat. The friend tells a teacher the next morning.
2. **At the same time:** The student's concern goes straight to the DSL. The teacher engages with the friend: thanks them, and tells them they did the right thing.
3. **At the same time:** The student is found and kept with a trusted adult. The friend is supported: it is not their job to keep their friend safe. The group chat is not investigated by staff.
4. **Joined up:** The DSL leads: a plan for the student, support for the friend, and a decision about the group chat, including whether other students need support. Contagion risk is considered.
5. **Every strand:** Every strand ends in Document. Who knew, and what was shared online, is part of the record.

When it breaks

The same Year 11 student, when steps are skipped.

1. **Turn 1:** The student says "I want to die" in a revision session. Recognise happens: the teacher hears it.
2. **Pause skipped:** The teacher says: "You don't mean that. Everyone's stressed. Take five minutes and come back in." The student goes to the toilets alone.

3. **Document skipped:** Nothing is recorded. The earlier concern about withdrawal and the line in the assignment are never connected.
4. **Notify skipped:** The DSL never hears. Professionals working in isolation, and information not being shared, are recurring findings when reviews look back at a child's death.
5. **The helix fragments:** The student has learned that saying it out loud changes nothing, and stops saying it. That is how risk grows unseen.
6. **Put it back together:** Every step taken puts the structure back. Stay with them, ask directly, tell the DSL. Relational is why young people tell; systemic is why telling works.

The Helix is described in the RESPOND Safeguarding Operating System (section 5.2). All examples are fictional.

R

RECOGNISE: Take It Seriously

When a student says they want to die, believe that they mean it enough to need help now. You don't need to decide how serious it is: you need to respond, and pass it on.

The 4 Ps Context Scan

People

Who is around the student?
Recent loss, family changes, a relationship ending, bullying, friends who are worried or who already know

Places

Exam rooms and revision sessions, toilets and quiet corners, the journey home, the boarding house at night

Platforms

Messages, posts or searches about suicide or self-harm; content they have seen; group chats; online contacts

Patterns

Withdrawal, poor sleep, falling work, giving up activities, earlier self-harm, pressure building before exams

What Students Might Say or Write

Direct

- "I want to die." "I want to kill myself."

- "I'm going to end it."
- "I've been thinking about suicide."

Less Direct

- "Everyone would be better off without me."
- "I can't see a way out." "What's the point?"
- "I won't be around to worry about results."
- "I'm a burden." "I just want it all to stop."

It may also appear in written work, artwork, a search history, a message to a friend, or a post. Treat it in the same way.

Warning Signs Staff Are Well Placed to See

KCSIE 2026 says that where a child is struggling with their mental health, self-harming or experiencing suicidal ideation, there are likely to be warning signs that education staff are well placed to recognise, including (paragraph 48):

- significant changes in behaviour
- ongoing difficulty sleeping
- withdrawing from social situations
- not wanting to do things they usually like
- physical signs of self-harm or neglecting themselves

Others you may notice: talking or writing about death, saying goodbye or giving things away, a sudden change in mood, more risk-taking, or saying they feel hopeless or trapped. Not every student who shows these signs is suicidal (paragraph 49), but each one is a reason to check in and to tell the DSL.

No Disclosure Needed

Many students will not say the words. If you notice warning signs, a worrying piece of writing, or a friend tells you they are worried, pass it to the DSL **without waiting for the student to tell you**. You don't need certainty, and you don't need to wait for it to happen again.

What the Guidance Says

- **KCSIE 2026, paragraphs 47 and 50:** see *Why This Matters* above. Mental health problems can also be an indicator that a child has suffered, or is at risk of, abuse, neglect or exploitation (paragraph 47).
- **Paragraphs 226 to 234:** schools support mental health through a whole-school approach, a mental health lead, early targeted support and referral to specialist services; the DSL considers whether to inform parents where doing so would not place the child at additional risk (paragraph 229).
- **NICE NG225** sets out how education settings should respond to self-harm, including support plans developed with mental health professionals, the student and their family (NICE, recommendations 1.8.3 to 1.8.7).

Source: Keeping Children Safe in Education 2026, checked September 2026. Staff don't diagnose, and don't decide on their own how serious the risk is.

What the Data Shows

7.4%

of 17-year-olds in the UK
said they had ever attempted
suicide

Patalay and Fitzsimons, 2021

24.1%

of 17-year-olds had self-
harmed in the previous 12
months

Patalay and Fitzsimons, 2021

1 in 5

8 to 16 year olds in England
had a probable mental
disorder in 2023 (20.3%)

NHS England, 2023

About half

of young people under 25
who died by suicide had
previously self-harmed

NCISH, 2017

Over a third

of children who died by
suicide had never been in
contact with mental health
services

NCMD, 2021

Millennium Cohort Study: 10,103 young people surveyed at 17 in 2018 to 2019. NHS survey carried out February to April 2023. NCMD reviewed child deaths by suicide in England from April 2019 to March 2020. Checked September 2026.

E

ENGAGE: Ask Directly and Listen

Stay with the student, stay calm, and ask the question directly. Use TED, never "why". You are listening to understand, not to fix.

What a Student Might Say

"I can't do this anymore. I want to die. I'm going to fail everything and everyone will see how useless I am."

"It doesn't matter. I won't be here for results anyway."

"Please don't tell anyone. I'm fine, honestly, I just panicked."

Ask Directly About Suicide

Use clear words. Asking does not put the idea in someone's head. Samaritans says that asking someone directly about suicide gives them permission to tell you how they feel, and lets them know they are not a burden (Samaritans); YoungMinds says talking about suicidal thoughts will not increase risk or put ideas in their head (YoungMinds).

"When you say you want to die, are you thinking about suicide?"

"Are you having thoughts of ending your life?"

"Have you done anything to hurt yourself today, or taken anything?"

"Are you safe right now?"

If the answer to any of these is yes, or you are unsure, treat it as **immediate risk**: stay with the student and get the DSL now. You don't need the details of any plan, and you shouldn't probe for them. The DSL and health professionals will ask what they need to.

When They Tell You

The RESPOND disclosure script:

"I believe you, and I'm taking what you've told me seriously. This is not your fault. I'm going to help keep you safe, and I'll explain what happens next."

- **TED:** "Tell me what's been happening." "Explain what's been making things feel this hard." "Describe how you've been feeling this week."
- Reflect what you hear: "It sounds like you've been carrying a lot on your own."
- Let silences happen. Keep your voice low and slow, and sit alongside rather than opposite, if that feels safer for them.
- Thank them: "I'm really glad you told me. That took courage."
- "Who else knows how you've been feeling?" (friends, family, anyone online)

Helpful

- "I'm here, and I'm staying with you."
- "You don't have to have the words right. Take your time."
- "Feelings this strong can change, and there are people who can help."
- "Thank you for telling me. You're not a burden."

Unhelpful

- "Why would you say that?" (a "why" question can sound like blame)
- "You've got so much to live for." (can leave them feeling unheard)
- "Think how your parents would feel." (adds guilt and shame)
- "You don't really mean that." (dismissive, and may stop them talking)
- "Let's keep this between us." (you can't, and it puts them at risk)

If the Student Is Panicking

Help them feel safe in their body first. Move to a quieter space if you can, sit down, and breathe slowly with them: in through the nose, and a longer breath out. Name what you see without judgement: "Your breathing is fast. That's your body trying to protect you. It will settle." Once they are calmer, ask directly about suicide. Don't assume a panic attack means the words didn't matter.

If Other Students Are Present

Protect the student's dignity. If it is safe to do so, use the 4Ds: **Distract** ("Can you help me with something outside for a moment?"), **Delegate** (ask a colleague to take the group, or to fetch the DSL), **Delay** (check in with the rest of the group afterwards), or **Direct** ("Everyone, give us some

space, please"). Don't let the student leave on their own, and don't leave them alone to deal with the group. Afterwards, tell the DSL which students saw or heard what happened.



SUPPORT: Safety First

Your first job is safety now. Never leave the student alone if there may be immediate risk. Your calm is contagious.

Immediate Safety

- **Stay with the student.** Send a colleague, or a reliable student, to fetch the DSL or a deputy. Use the phone or radio if you have one. Don't send the student to find the DSL on their own.
- **Move somewhere quiet and safe**, with another adult nearby. Keep the door open or ajar, and don't block the way out.
- **If they have hurt themselves, or have taken anything:** get first aid and the school nurse straight away.
- **Call 999** if a life is at risk: a serious injury, if they have taken an overdose or swallowed something harmful (even if they seem well), if they are unconscious, or if they cannot keep themselves safe. Don't wait for the DSL to do this.
- If the student tries to leave while at immediate risk, don't put yourself or others in danger or chase them into danger. Physical intervention is a last resort, only where it is reasonable and proportionate to stop serious harm, and in line with the school's policy (KCSIE 2026, paragraph 205). Keep them in sight if you safely can, tell the DSL immediately, and call 999.

Medical Attention

NICE NG225 says professionals in non-health settings should address any immediate physical health needs from self-harm, in line with local policies, and call 111 or 999 if necessary (recommendation 1.8.1). The NHS advises calling 999 or going to A&E now if someone's life is at risk, for example if they have seriously injured themselves or taken an overdose, or if they don't feel they can keep themselves safe (NHS).

- Use the school's first aid and medical procedures. Wear gloves for any wound care, as you would for any injury.
- Don't ask the student to show you injuries that are covered. Let the nurse or first aider assess them in private.
- Keep your reaction calm and matter-of-fact. Shock or disgust can increase shame.

Immediate Support

- Water, tissues, a quiet space, time. Don't send them back to the revision session or to their next lesson.
- "You're not in trouble. You've done the right thing by telling me."
- "You don't have to work this out on your own. I'm going to stay with you while we get the right person."
- Ask what would help right now: some students want to talk, others want quiet company.
- Keep the student with a trusted adult until the DSL has taken over and a plan is in place.

SEND, Neurodivergence and Language

- Allow extra processing time. Use clear, concrete language and one question at a time.
- Some autistic students may speak very directly about death, or show distress through shutdown rather than tears. Take what they say literally and ask directly.
- Offer other ways to communicate: writing, drawing, a feelings scale, or a trusted adult who knows them well.
- Reduce sensory load: lower lights, fewer people, less noise.
- Use a professional interpreter where language is a barrier, never another student or a family member.
- Record any adjustments you made.

Support for Others Involved

Students who were in the room, or friends who have been told, may be shaken. Make sure an adult checks on them. Don't share details. Tell them: "Your friend is being looked after by the right people. It's OK to feel upset. Who can you talk to today?" Pass their names to the DSL.



PAUSE: A Breath, Not a Delay

Where there is immediate risk, the Pause is a breath while you act, not time spent deciding. Where there is not, take 90 seconds: breathe, lower your shoulders, and check your own state before you decide what to say.

Three Lenses

Lens	In this scenario
Risk	Is there immediate risk? Have they hurt themselves or taken anything? Do they have plans? Are they about to be alone, go home to an empty house, or leave the site?
Rights	The student's right to be safe, to be listened to and taken seriously, to dignity and privacy, and to a say in what happens next
Resources	The DSL and deputies, the school nurse, the mental health lead and counsellor, NHS 111 (mental health option), 999, parents and carers, and trusted adults in the student's life

What You Must Not Do

Don't Leave Them Alone

- Not to make a phone call, fetch a colleague or finish the session
- Not in a toilet, a corridor or an empty room
- Not to walk to the DSL's office on their own

Don't Promise Secrecy

"I can hear you don't want anyone else to know, and I understand. I can't keep this to myself, because your safety matters too much. I'll speak to the DSL, whose job is to get you the right help, and I'll tell you what's happening at each step."

Don't Investigate

- Don't search the student's bag, room or phone, or ask friends to find out more
- Don't ask for details of methods or plans beyond what you need to know that there is immediate risk
- The DSL and health professionals assess risk; you pass on what you know

Don't Judge or Debate

- Don't call it attention-seeking or dramatic
- Don't argue with them about whether life is worth living
- Don't lecture, or make them promise not to do anything

Check Your Own Response

- Notice your own reaction: fear, panic, the urge to fix it, or the urge to play it down. All are normal. Your calm is contagious.
- Avoid the rescuer mindset: you are the bridge to help, not the whole answer.
- Be aware of how your own experiences of suicide, self-harm or loss may shape your response.
- If you feel overwhelmed, keep the student safe first, then say: "This is important and I want to get it right. I'm going to stay with you while someone gets the DSL."

OFFER: Model Alternatives

Offer hope and choices without pressure, explain what happens next, and model that feelings this strong can be talked about and can change. If in doubt, N before O: tell the DSL before offering options.

Explain What Happens Next

"Here's what happens now. I'm going to stay with you, and our safeguarding lead is coming to see you. Their job is to make sure you're safe and to get you the right help, which might include the school nurse, the counsellor, or a mental health service. Because you're under 18 and we're worried about your safety, your parents or carers will usually need to know; we'll talk with you about how. You're not in trouble. What questions do you have?"

Be honest about information sharing. Staff should share safeguarding information as early as possible, and data protection law does not prevent it; safeguarding children is a lawful basis for sharing sensitive information without consent where appropriate (KCSIE 2026, paragraphs 142 to 148). What is shared must be relevant, necessary and proportionate, go only to those who need it, and be handled in line with data protection law. From 30 September 2026, the Information Sharing Duty (section 16LA, Children Act 2004) also requires the organisations it covers to share where its tests are met; the DSL

decides how it applies. So don't ask for consent as if it were the legal basis. Tell the student what will be shared, with whom and why, unless that would increase the risk, and listen to their views: they shape the plan. Agreeing to a service, such as counselling, is a separate question, and their agreement matters there.

Options, Not Orders

- "Would you like me to stay while you talk to the DSL?"
- "Is there someone at school you'd like with you?"
- "How would you like your parents to be told: with you there, or with us leading?"
- "What has helped you get through hard moments before?"
- "What would make tomorrow at school feel more manageable?"

Some things have to happen to keep them safe. Where there is a choice, give it to them.

Model the Alternative

What You Can Model

- Talking openly about suicide is safe, and it helps
- Feelings this intense can pass, even if it doesn't feel like it now
- Asking for help is a strength
- Exams matter, and there is always another route

What the Student May Believe

- "No one can help."
- "I'm a burden."
- "If I fail these exams, my life is over."
- "Telling will make it worse."

Use the student's own words to reflect a different view, gently. Don't argue: show them, by how you respond, that telling someone was the right thing to do.

Support in School and Beyond

- The school nurse, counsellor or mental health lead; a daily check-in with a trusted adult
- The tutor and key teachers made aware (without details) so they can support
- A safety plan, developed with the student and, where appropriate, their family and health professionals
- Childline (for anyone under 19): 0800 1111, free and confidential; call any time, day or night, or use 1-2-1 chat at [childline.org.uk](https://www.childline.org.uk) (you may have to wait for a chat counsellor, so calling is usually quickest)
- Shout: text **SHOUT** to 85258, free, confidential, 24/7
- Samaritans: 116 123, free from any phone, any time, day or night, whatever your age
- NHS 111: call 111 and choose the mental health option (option 2), for urgent mental health help in England, any time, or use NHS 111 online
- Stay Alive app (Grassroots Suicide Prevention): free, with a personal safety plan and a directory of help

N NOTIFY: Tell the DSL Now

Where there may be immediate risk, tell the DSL immediately, and stay with the student while you do. In every other case, tell the DSL as soon as is practicably possible, and the same day. If a life is at risk, call 999.

Who and When

- **Immediate risk:** the DSL or a deputy, now, by phone, radio or a colleague. Don't leave the student to do it.
- **Life at risk:** call 999 first, then the DSL.
- **No immediate risk** (for example, a worrying line in written work): the DSL, as soon as is practicably possible, and before the student goes home.
- If the DSL and deputies cannot be reached, go to a senior leader. Anyone can refer directly to children's social care if they believe a child is at risk; telling the DSL does not remove your own responsibility.

What to Tell the DSL

- Exactly what the student said, in their words, and when
- Whether they have hurt themselves or taken anything, and any first aid or medical help given

- Whether they said they had thoughts of suicide, and anything they said about plans or being unsafe
- Where the student is now, and who is with them
- Who else knows, including friends, and anything online
- What you have said and done so far, including anything you told them would happen
- Anything you know about pressures: exams, loss, bullying, home, relationships

▼ For the DSL: assessment, external agencies and parents

Most staff need only: tell the DSL, today, with the information above.

The DSL's Initial Actions

- Make sure the student is safe and not alone, and that any physical health needs are met
- Seek mental health advice: NHS 111 (mental health option) for urgent help, or 999 and A&E where the student is in danger (KCSIE 2026, paragraph 50). Follow the local crisis pathway where one exists
- Decide who contacts parents or carers, when and how, considering whether this could place the student at additional risk (KCSIE 2026, paragraph 229)
- Make sure the student goes home, or stays in school or the boarding house, to a safe adult and a plan for tonight
- Consider whether abuse, neglect or exploitation may sit behind the distress (KCSIE 2026, paragraph 47), and refer to children's social care where needed
- Refer to local children and young people's mental health services, with parents and the GP as appropriate; involve the MHST where the school has one
- Develop a support plan and in-school safety plan with the student, their family and mental health professionals (NICE NG225, recommendation 1.8.6). Don't rely on risk assessment tools or scores to predict risk (recommendation 1.6.1)
- Identify and support friends and peers who know or are affected, and consider contagion risk
- Apply the Information Sharing Duty: share what is relevant and proportionate with those who need it; record the decision to share or not share, and be transparent with the student
- Record on the safeguarding system, with a chronology

Routes to Help

Route	When
999 (ambulance)	A life is at risk: serious injury, an overdose or anything harmful swallowed (even if they seem well), or a student who cannot keep themselves safe

Route	When
A&E	A child is in danger: call 999 or arrange for them to be taken to A&E immediately by a parent, carer or other suitable person (KCSIE 2026, paragraph 50)
NHS 111, mental health option	Urgent mental health help that is not an emergency: call 111 and choose the mental health option (option 2), any time, or use NHS 111 online (KCSIE 2026, paragraph 50)
Local children and young people's mental health services (CYPMHS, often still called CAMHS)	Assessment and treatment; referral by the GP, school or another professional (self-referral in some areas)
GP	Care of injuries, review and referral; usually arranged with parents
Mental Health Support Team (MHST)	Early support and advice, where the school has access (KCSIE 2026, paragraph 232)
Children's social care	Signs of abuse, neglect or exploitation behind the distress (KCSIE 2026, paragraph 47); where telling parents could increase the risk; or where parents cannot keep the child safe
Police	A student who is missing and at risk, or immediate danger to anyone

Where a Student Is 18

At 18 a student is legally an adult. The child safeguarding framework in KCSIE applies to under-18s, but the school still has welfare responsibilities. Where an 18-year-old is involved, the DSL considers adult safeguarding, police and welfare pathways alongside the school's procedures, and the adult student's own views and consent in a different way. This matters in sixth form and in boarding, where students turn 18 during their final year.

Parents and Carers: Usually Yes, but the DSL Decides

Parents and carers are usually a key part of keeping a child safe, and they need to know when there is risk. KCSIE 2026 says the DSL should consider whether to inform parents or carers to support the child's wellbeing, **where doing so would not place the child at additional risk** (paragraph 229). Where the student says something at home is part of the reason, where there are signs of abuse or neglect, or where honour-based abuse is possible, the DSL seeks advice from children's social care before contacting parents and agrees a safe plan. Tell the student honestly that parents will usually need to know, and ask how they would like it done. Don't promise that parents won't be told.

Parent contact: what to say

- "I'm calling because [name] told a member of staff today that they have been having thoughts of wanting to die. They are safe and with [staff member] now."
- "[Name] did the right thing by telling us. Please stay calm with them, listen, and don't be angry. It may help to keep medicines and anything that could be used to cause harm safely stored at home."
- "Here are the numbers for tonight: NHS 111 option 2, Samaritans on 116 123, and 999 in an emergency."

D DOCUMENT: Record Observations

Record as soon as is practicably possible, on the school's safeguarding system, using **GOLD**. Where there is immediate risk, safety and the DSL come first; write it up once the student is with the right people.

GOLD

Ground

Date, time, place, who was present, who you are, and how it came to your attention

Observe

What you saw and heard, with exact words in quotation marks

Language

Neutral, factual and non-diagnostic. No opinions or labels

Do

What you did, what you said would happen next, and why

Example: Exact Words

- Student said: "I can't do this anymore, I want to die."
- When asked "Are you thinking about suicide?", student said: "Sometimes. At night, mostly."
- When asked if they had hurt themselves or taken anything today, student said: "No."
- Observed: crying, breathing fast, hands shaking; calmer after about ten minutes of slow breathing in the side room

- Actions: stayed with the student; a colleague sent for the DSL at 4:10pm; DSL arrived 4:15pm and took over

Recording template

SAFEGUARDING CONCERN: SUICIDAL THOUGHTS

Date and time:

Student: name, year, tutor group

Recorded by: name and role

Where and how it came to your attention:

WHAT THE STUDENT SAID (exact words in quotation marks):

QUESTIONS I ASKED AND THEIR ANSWERS (exact words):

INJURY OR ANYTHING TAKEN: yes / no / not known; first aid or medical help given:

OBSERVATIONS (presentation, emotional state):

WHO ELSE KNOWS (friends, family, online):

WHAT I DID AND SAID (with times):

SHARED WITH: DSL (name, time, how); 999 called: yes / no, time

THE STUDENT'S WISHES AND VIEWS:

Signed and dated:

What Not to Record

- Opinions or labels such as "attention-seeking", "dramatic" or "manipulative"
- A diagnosis, or your own guess about how serious the risk is
- Speculation about the cause, or about the family
- Details you didn't hear from the student. Record what they said in their words, and give anything about plans or means to the DSL directly

Return After a Crisis

Coming back can feel as hard as leaving. Plan it, and keep it under review.

A student may return after time at home, a hospital attendance or a mental health assessment. The first days back matter. The DSL or mental health lead plans the return **with** the student and their parents or carers, and with health professionals where they are involved (NICE NG225, recommendation 1.8.6).

- A short meeting before or on the first morning: what the student wants people to know, and what they don't

- One named key adult and a regular check-in, including a way to find them
- A discreet way to leave a lesson and a safe place to go, agreed in advance
- The in-school part of any safety plan: warning signs, what helps, who to go to, and what staff will do
- Adjustments to workload, deadlines and exams, reviewed regularly
- A review date, and a plan for holidays, weekends and exeats when school support is not there

Safety Plans

A safety plan is developed with the student, and with their family where appropriate. It covers warning signs, what helps, people and places that feel safe, and who to contact in a crisis (NICE NG225, recommendations 1.11.7 and 1.11.8). Health professionals lead the plan where they are involved; the school holds the in-school part. Some students keep theirs on their phone, for example in the free Stay Alive app. NICE NG225 advises against using risk assessment tools and scales to predict future suicide or repetition of self-harm (recommendation 1.6.1): focus on needs and support, not a score.

Exam Pressure

Exams bring a concentrated period of pressure, often with less sleep, less exercise and less time with friends. Staff who teach and invigilate are often the first to see a student struggling.

- Academic pressure is one of the stresses reported more often in young people under 20 who died by suicide, alongside bullying (NCISH, 2017). It is rarely the only factor: suicide is complex, and no single cause explains it.
- Notice the build-up: mocks, predicted grades, deadlines, the exam season and results days. Some students hide distress behind high achievement.
- Never send a student in distress back into an exam or revision session to "get it done". Safety comes first. Tell the exams officer that the student needs support, without sharing details they don't need.
- The DSL, the exams officer and the SENCO can consider access arrangements, a separate room, rest breaks, rescheduled mocks or an application for special consideration, following the exam board's rules.
- Talk about exams in a way that keeps things in proportion: "Results matter, and you matter more. There is always another route."



Peers, Online and Contagion Risk

Self-harm and suicidal thoughts rarely affect one student alone. Friends, group chats and online spaces are part of the picture.

Friends Who Know

- Students often tell a friend first. Ask the student, gently: "Who else knows about this?" Pass any names to the DSL.
- Friends who have been told may be frightened, carrying a secret, or feeling responsible. The DSL decides who speaks to them. Thank them, tell them they did the right thing, and make it clear it is not their job to keep their friend safe.
- NICE NG225 recommends that education settings recognise the effect of a student's self-harm on their peers and provide support to reduce distress to them and to the student (recommendation 1.8.7).

Online

- Posts, messages or searches about suicide or self-harm, including content a student has seen or shared, go to the DSL straight away. Don't look through a student's phone yourself.
- KCSIE 2026 lists content about self-harm and suicide among the online content risks (paragraph 165). Harmful content can be reported to the platform; the DSL decides what else is needed.
- If a student posts something that suggests immediate risk, treat it as immediate risk: tell the DSL immediately, or call 999 if you cannot reach them and a life may be at risk.

Contagion and Clusters

- Young people are more susceptible to suicide contagion (Samaritans Media Guidelines, 2026). When a student harms themselves, attempts suicide, or dies by suicide, others close to them, or who identify with them, may be at greater risk (Public Health England, 2019).
- Don't discuss the details of what a student did in class, assemblies or messages to parents. Keep communication factual, brief and focused on support, and agree it with the DSL and senior leaders.
- After a death, avoid anything that could glamorise it or present suicide as a solution. Plan memorials carefully, keep routines as normal as possible, and make support visible.
- Samaritans Step by Step supports schools after a suspected or attempted suicide, free of charge: freephone 0808 168 2528, or contact the team online.

Safe Language

Say "died by suicide" or "took their own life", never "committed suicide". Avoid "successful" or "failed" attempt. Don't describe methods, don't speculate about a single cause, and always point to where help is available (Samaritans Media Guidelines). Never describe self-harm as attention-seeking.

Boarding and Residential Life

For boarders, school is also home. Distress often peaks in the evening and at night, and families may be far away.

Step	In boarding
● Recognise	Use the 4 Ps for the house: who shares a room, who they eat and study with, evenings and weekends, phones after lights out. Notice changes at meals, bedtime and sleep.
● Engage	House staff often hear things first, late at night. Ask directly about suicide, calmly. Talk in a visible space with another adult aware.
● Support	Safety tonight comes first: never leave a boarder alone if there may be immediate risk. Who is on duty, where will they sleep, and how do they reach you?
● Pause	Consider the whole house. Don't search rooms, investigate or promise secrecy. If there may be immediate risk, don't wait until morning.
● Offer	Model alternatives: evenings with company, routines that help sleep, and a clear way to find staff at night. If in doubt, N before O.
● Notify	Contact the on-call DSL straight away where there may be risk. If a life is at risk, call 999; out of hours, the DSL may also contact the local authority's emergency duty team.
● Document	Record on the safeguarding system before you go off duty, using GOLD. A house log or handover note is not a substitute.

Night-Time and the Boarding House

- Evenings and nights can be when students feel worst and when fewer adults are around. Every boarder should know how to reach the member of staff on duty at night.
- Where the DSL and health team decide a student needs closer observation overnight, they set out what, how often and by whom. Checks should be proportionate, respectful and explained to the student; they should not become public or punitive.

- The DSL and the health team decide how access to anything that could be used to cause harm is managed for a student at risk. House staff follow that plan and don't make their own searches.
- Room-mates and close friends in the house may know, or may be affected. The DSL decides what they are told and makes sure they are supported.
- If a boarder may be at immediate risk at night, don't wait until morning: contact the on-call DSL, and call 999 if a life may be at risk.

Boarders' Routes to Help

- **Boarders can always contact their house parents, the DSL or a member of the boarding management team, at any time, day or night.** Make sure every boarder knows how.
- An independent person outside the staff whom boarders can contact directly (NMS 11.4), often called the independent listener
- Boarders must be able to contact their families privately; monitoring of communications should be proportionate (NMS 12.1)
- International boarders: agree with the DSL who contacts parents or guardians, when, and in what language, using interpreters where needed

Source: National Minimum Standards for Boarding Schools (2026).

Identity and Intersecting Risk

Any student can have suicidal thoughts or self-harm. Some groups face additional pressures that can raise risk or make it harder to ask for help. This is about their experiences, not who they are. Use this to notice and to ask, never to assume.

- **LGBTQ+ students:** in a study of more than 10,000 17-year-olds in the UK, sexual minority young people reported much higher rates of self-harm in the past year (55.8% compared with 20.5%) and of ever having attempted suicide (21.7% compared with 5.8%) (Patalay and Fitzsimons, 2021). KCSIE 2026 recognises that prejudice-based harm can affect a child's mental health and willingness to seek help (paragraph 17). Don't assume a student's identity is the cause, and don't share it with anyone, including parents, without the DSL's advice.
- **Neurodivergent students:** in a national review of child deaths by suicide, 16% of the children had a confirmed diagnosis of a neurodevelopmental condition such as autism or ADHD (NCMD, 2021). Distress may look different: shutdown, changes in routine or special interests, or very literal statements. Take literal statements literally, and ask directly.

- **Care-experienced students**, and those with a social worker, may have experienced loss, trauma and changes of placement. Involve the designated teacher, the social worker and the Virtual School Head through the DSL.
- **Bereavement and loss**: 62% of the children in the same national review had experienced a significant personal loss, and almost a quarter had experienced bullying (NCMD, 2021).
- **International students and those with English as an additional language** may come from settings where mental health is rarely talked about, or worry about what their family will think. Use a professional interpreter where needed, never another student or a family member.

Looking After Yourself After a Disclosure

Your wellbeing matters, and a supported adult is better able to support a student.

- Hearing a student say they want to die, or seeing injuries, is distressing. Feeling shaken, tearful, angry or numb afterwards is a normal response.
- Ask the DSL for a short debrief, and to be told, as far as appropriate, what happens next. Not knowing can be hard.
- If it connects with your own experience, or someone close to you, talk to your line manager, a colleague you trust, the school's Employee Assistance Programme if it has one, or Education Support.
- If it stays with you (poor sleep, going over it again and again), ask for support early.
- Education Support: 08000 562 561, free, confidential, 24/7, for education staff
- Samaritans: 116 123, free from any phone, any time, day or night, whatever your age, for staff too

Additional Resources and Support

For Students

- Childline (for anyone under 19): 0800 1111, free and confidential; call any time, day or night, or use 1-2-1 chat at [childline.org.uk](https://www.childline.org.uk) (you may have to wait for a chat counsellor, so calling is usually quickest). **For under-18s, this is the place to start.**
- Shout: text **SHOUT** to 85258, free, confidential, 24/7
- Samaritans: 116 123, free from any phone, any time, day or night, whatever your age
- NHS 111: call 111 and choose the mental health option (option 2), for urgent mental health help in England, any time, or use NHS 111 online

- Stay Alive app (Grassroots Suicide Prevention): free, with a personal safety plan and a directory of help

For Parents and Carers

- YoungMinds Parents Helpline: 0808 802 5544 (Monday, Thursday and Friday 9:30am to 4pm; Tuesday and Wednesday 9:30am to 6pm)
- NSPCC Helpline: 0808 800 5000 (Monday to Friday 10am to 4pm), or help@nspcc.org.uk
- YoungMinds, *Suicidal thoughts: a guide for parents*
- YoungMinds, *Self-harm: a guide for parents*

For Staff

- KCSIE 2026: paragraphs 47 to 50 and 226 to 234 (mental health)
- Samaritans, *Supporting someone with suicidal thoughts*
- Samaritans, *Media Guidelines for Reporting Suicide* (2026)
- Samaritans, *Step by Step*: support for schools after a suspected or attempted suicide
- Public Health England, *Identifying and responding to suicide clusters: a practice resource* (2019)
- Education Support: 08000 562 561, free, confidential, 24/7, for education staff

Papyrus HOPELINE247 Has Closed

Papyrus Prevention of Young Suicide entered administration in September 2026 and its services, including HOPELINE247, are no longer operating (South West Yorkshire Partnership NHS Foundation Trust, September 2026). Check posters, planners, websites and wellbeing pages for the old number and replace it with the routes in this guide.

Statutory Guidance

- Department for Education, *Keeping Children Safe in Education 2026*
- HM Government, *Working Together to Safeguard Children 2026*
- Department for Education, *Information Sharing Duty: statutory guidance* (in force 30 September 2026)
- Department for Education, *RSHE statutory guidance* (published July 2025; statutory from September 2026)
- Department for Education, *National minimum standards for boarding schools* (2026)
- Department for Education, *Promoting and supporting mental health and wellbeing in schools and colleges* (updated April 2026)

Health and Clinical Guidance

- NICE, *Self-harm: assessment, management and preventing recurrence* (NG225), recommendations
- NHS, *Where to get urgent help for mental health*
- NHS England, *Children and young people's mental health services (CYPMHS)*
- YoungMinds, *Guide to CAMHS*
- Evelina London Children's Hospital (Guy's and St Thomas' NHS Foundation Trust), *Staying safe: information for parents*
- Childline, *Self-harm: ways to get support*

Figures

- Patalay and Fitzsimons, *Psychological distress, self-harm and attempted suicide in UK 17-year olds*, British Journal of Psychiatry (2021), Millennium Cohort Study
- NHS England, *One in five children and young people had a probable mental disorder in 2023*
- National Confidential Inquiry into Suicide and Safety in Mental Health, *Suicide by children and young people* (2017)
- National Child Mortality Database, *Suicide in children and young people* (2021)

Contacts, figures and references checked 28 September 2026. Verify at point of use.



Quick Reference Card

Immediate Danger Comes First

Immediate danger or possible risk to life: call 999 when it is needed, stay with the child, and summon the DSL or a deputy. Don't delay action to work through the steps. The 90-second Pause is for once immediate safety is secured, to stop an impulsive or investigative response.

Recognise

- "I want to die", in words, writing or online: take it seriously
- Withdrawal, poor sleep, hopelessness, giving things away, signs of self-harm
- No disclosure needed, and no need to wait for a pattern

Respond

- Stay with them if there may be immediate risk
- Ask directly: "Are you thinking about suicide?"
- "I believe you, and I'm taking what you've told me seriously. This is not your fault."
- TED, never "why". Don't promise secrecy

Notify

- Immediate risk: DSL now. Life at risk: 999
- Otherwise: DSL as soon as is practicably possible, the same day
- Injury or anything taken: first aid, nurse, 999 if serious
- If in doubt, N before O

Document

- GOLD: Ground, Observe, Language, Do
- Their exact words, your questions and their answers
- No labels, no diagnosis, no speculation

Key Numbers

- Emergency: 999
- NHS 111: call 111 and choose the mental health option (option 2), for urgent mental health help in England, any time, or use NHS 111 online
- Samaritans: 116 123, free from any phone, any time, day or night, whatever your age
- Childline (for anyone under 19): 0800 1111, free and confidential; call any time, day or night, or use 1-2-1 chat at [childline.org.uk](https://www.childline.org.uk) (you may have to wait for a chat counsellor, so calling is usually quickest)
- Your DSL: named in the school's safeguarding policy

Online Evidence: One Rule

- **Ordinary messages or posts a student has already shown you:** note what is relevant (platform, date, who, exact words) in your record. Don't ask to see more or scroll through a phone, especially where messages may be sexual, because they may contain images. Don't log in to accounts, search profiles or investigate, and never transfer anything to a personal device.
- **Nude or semi-nude images of anyone under 18:** don't intentionally view, copy, screenshot, forward, save or delete them. Tell the DSL straight away. If you see one by accident, tell the DSL, ask for support if you need it, and record that it was seen and what you did, without describing the image (UKCIS).
- The DSL decides what happens to devices and content.

Your School's Contacts

This guide is generic. Schools add their own details here, or keep a completed copy with this card, so that anyone responding, including at night, has the routes to hand.

Contact	Name and how to reach them
Designated Safeguarding Lead (DSL)	
Deputy DSL(s)	
Out-of-hours and boarding on-call safeguarding contact	
How to record a concern (system or form)	
Local authority children's services front door	
Out-of-hours emergency duty team	
LADO (concerns about adults)	
Police: 999 (emergency); 101 (non-emergency)	

RESPOND Safeguarding Framework Reference Scenarios · Staff Guide 34

Scenario 34: Student Expresses Suicidal Thoughts

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Aligned with: KCSIE 2026 | Working Together to Safeguard Children 2026 | Information Sharing Duty (September 2026) | NICE NG225 | RSHE statutory guidance (published July 2025; statutory from September 2026) | NMS for Boarding Schools 2026 | Samaritans Media Guidelines

Last Updated: September 2026 | Next Review: September 2027, or on publication of KCSIE 2027 if sooner

For staff in schools and other settings working with children. You may share it with colleagues. Share the student and parent guides, not this page, with students and families.